

Section GG Scoring Guide

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The Section GG Rating Scale

Code	Rating	What It Means
06	Independent	Patient completes activity safely with no assistance, setup, or supervision. No helper needed.
05	Setup or Clean-Up Assistance	Helper sets up equipment, positions items, or cleans up afterward — but does not assist during the activity itself.
04	Supervision or Touching Assistance	Helper provides verbal cues, safety monitoring, or touching/guiding without providing physical assistance. Patient does >98% of effort.
03	Partial/Moderate Assistance	Helper provides less than half the effort. Patient does 50–98% of the work.
02	Substantial/Maximal Assistance	Helper provides MORE than half the effort. Patient does 25–49% of the work.
01	Dependent	Helper does all or almost all of the effort. Patient does less than 25% — or the activity does not occur.
07	Patient Refused	Activity was not attempted because patient refused. Not the same as unable.
09	Not Applicable	Activity does not occur in the SNF setting (e.g., patient does not walk at home).
10	Not Attempted — Safety Concern	Clinician determined activity unsafe to attempt given patient's condition.
88	Not Attempted — Other Reason	Activity not attempted for another reason not listed above.

Section GG Items — What Is Being Scored

Item #	Category	Activities Scored
GG0100	Self-Care	Eating, oral hygiene, toileting hygiene, shower/bathe self, upper body dressing, lower body dressing, put on/take off footwear
GG0110	Mobility — Bed/Chair/Wheelchair	Roll left/right, sit to lying, lying to sitting, sit to stand, chair to bed, toilet transfer, car transfer, WC to chair
GG0170	Mobility — Walking	Walk 10 ft, 50 ft (with/without turn), 150 ft, walking on uneven surfaces, 1 step curb, stairs (4 and 12 steps)
GG0170 WC	Mobility — Wheelchair (if applicable)	WC to bed (if patient does not walk — scored instead of walking items)

Scoring Examples — Common Scenarios

Activity	Scenario	Score & Rationale
Eating	Patient feeds self independently but needs someone to open containers and set up tray.	05 — Setup only. Patient does all eating without physical help.
Eating	Patient needs hand-over-hand guiding and verbal cues to initiate and complete meals.	03 — Partial assistance. Therapist provides less than half the effort but patient needs physical guidance.
Sit to Stand	Patient stands independently but needs verbal cue for foot placement each time.	04 — Supervision/touching. Verbal cue only, no physical assist.
Sit to Stand	Patient requires therapist to lift under bilateral arms and provide 50% of effort to achieve standing.	02 — Substantial assistance. Helper provides more than half.

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Activity	Scenario	Score & Rationale
Walk 50 ft	Patient ambulates 50 ft with FWW, CGA, and verbal cues for step length.	04 — Supervision/touching. CGA = touching without force.
Walk 50 ft	Patient ambulates 50 ft with 1-person contact assist for balance — therapist provides light support at trunk.	03 — Partial assistance. Therapist provides some physical support but patient does majority.
Lower Body Dressing	Patient requires max assist for lower body dressing due to hip precautions post-ORIF.	02 — Substantial/max assistance. Helper does most of the work.
Shower/Bathe	Patient requires total assist for shower — does not participate.	01 — Dependent. Patient contributes less than 25%.
Stairs (4 steps)	Patient does not attempt stairs — unsafe given current balance deficits.	10 — Not attempted due to safety concern.
12-step stair climb	Patient lives in a single-story home — stairs not applicable to discharge environment.	09 — Not applicable.

PDPM Impact — Why Accurate GG Scoring Matters

• GG scores at admission drive PDPM PT and OT component payment.	Lower scores (more dependent) = higher PDPM payment for PT/OT. Accurate scoring ensures appropriate reimbursement.
• GG discharge scores are required for QRP (Quality Reporting Program).	Missing or inaccurate discharge GG scores can result in a 2% Medicare payment penalty for the facility.
• GG scores should reflect observed performance, not what the patient CAN do.	Score what the patient ACTUALLY does during the assessment — not their best or worst. Use 3-day assessment window.
• Therapy and nursing must agree on GG scores — interdisciplinary process.	MDS coordinator collects GG data from therapy. Communicate your scores clearly and in writing before the MDS is finalized.
• Document the basis for each score.	Note the date, activity, conditions, and assist level observed. This supports the score if questioned on audit.

The 3-Day Rule: GG admission scores must be based on the patient's performance during the 3-day assessment window (typically days 1–3 of the stay). Discharge scores are assessed during the last 3 days before discharge. Do not extrapolate — base scores only on what you directly observe or what is clearly documented by another clinician during that window.